

Medicine of Potential

Body, diagnosis, treatment, and a person who is not a case

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Medicine tests the theory where potential is compressed into pain, symptom, appointment, scan, protocol, and risk. The body is not an idea and not a metaphor. It is the site where possibility may open, close, fracture, or remain suspended. Medicine of potential is not “soft” medicine against scientific medicine; it is the attempt to keep science in right relation to a living person.

The deep failure of medicine is not only diagnostic error or systemic overload. It is the replacement of the patient by the case. A person becomes cardiac, oncological, psychiatric, high-risk, a diagnostic code. Some reduction is necessary because systems must act. But the danger begins when the reduction starts to look like the whole truth. Categories are handles for action, not the person.

Responsible health = living body × accurate diagnosis × listening × evidence × treatment time × continuity × consent / overload × dehumanization × blind protocol × fear × commercialization × unsupervised automation. This is not a medical formula. It is a boundary map: when does protocol save, and when does it replace judgment; when does measurement illuminate, and when does it hide the person?

AI in medicine is the sharp example. A system can read images, summarize a chart, suggest differentials, flag drug interactions, estimate risk, and reduce burden. These can be valuable. But when the tool receives an aura of certainty, the body becomes a set of signals and the clinician becomes an approver of a conclusion. The problem is not using artificial intelligence. The problem is erasing the horizon of responsibility.

A simple case: pain that does not appear on a test. Bad medicine says: no finding, therefore no problem. Good medicine says: no finding within this current horizon of examination. At the same time, not every sensation is a diagnosis. The medical optimum is between two failures: cancelling the person in the name of measurement and cancelling measurement in the name of the person.

Medicine of potential links to boundary horizons because every diagnosis is a horizon: what can be seen, what remains hidden, and what is dangerous to infer. It links to the recursive edge because every successful treatment opens the next question: rehabilitation, quality of life, prevention, living with illness, or systemic change. It links to AI because a tool is morally useful only when it returns responsibility rather than stealing it.

This chapter gives no medical advice. It clarifies the condition of responsibility: the patient is not the output of a test, the physician is not a relay for an algorithm, and a system may not call efficiency “care” when efficiency was bought by erasing listening.

Source discipline

Working sources and caution: this chapter relies on the theory itself and on background sources checked outside the prompt: UNESCO on artificial intelligence in education, WHO on ethics and governance of AI for health, the NIST AI RMF for AI risk management, CERN/ATLAS on the Standard Model and the Higgs, NASA and ESA/Planck on cosmology, the Stanford Encyclopedia of Philosophy on Bell, Kochen-Specker and contextuality, and Britannica/ISFDB for the literary identification of Harlan Ellison and *I Have No Mouth, and I Must Scream*. These sources are not treated as proofs of the theory; they are safeguards against empty borrowing of terms.